AO-03 OB-GYN Emergency

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | October 16, 2026

Scope: Use for pregnancy-related or gynecologic hemorrhage, ectopic pregnancy, miscarriage, preeclampsia/eclampsia, trauma, syncope, shock, or other time-critical maternal emergency.

Maternal emergency, hemorrhage, and eclampsia pathway

1. Pregnancy/GYN emergency	Possible pregnancy, recent delivery, vaginal bleeding, abdominal pain, hypertension, seizure, syncope, or shock.
2. Position + assess	Left lateral when advanced pregnancy or supine hypotension is possible. ABCs, glucose, serial vital signs, bleeding amount, neurologic findings, and fetal/delivery history.
3. Shock/hemorrhage?	Control external bleeding, establish access, use AM-05 resuscitation, and consider TXA for qualifying postpartum hemorrhage.
4. Field delivery?	Open AO-01. After placental delivery, address uterine atony and postpartum bleeding.
5. Seizure/eclampsia?	Protect airway, start magnesium as cause-directed therapy, and use standardized seizure treatment for an ongoing convulsion.
6. Rapid transport	Notify an emergency/OB-capable destination early. A pregnancy with any chance of delivery remains ground transport; do not place on aircraft.

Key treatment and disposition

Situation	Approved action
Postpartum hemorrhage	After placental delivery, perform uterine massage. For hemorrhage with shock within 3 hours of delivery, give TXA 1 g IV/IO over 10 minutes.
Eclampsia	Magnesium sulfate 4-6 g IV over 20-30 minutes, then 1-2 g/hr infusion. Continuously monitor ECG, BP, respiratory status, SpO2, and EtCO2.
Ongoing convulsion	Use the standardized adult-seizure regimen: 10 mg IM when IV/IO is not established, 10 mg IN alternative, or 5 mg IV/IO; repeat 5 mg IV/IO once after 5 minutes if needed (maximum total 10 mg).
Magnesium toxicity	Stop magnesium for clinically significant toxicity. With Medical Control, give calcium gluconate 1 g IV slowly and support ventilation/hemodynamics.

AM-04's undifferentiated-CVA labetalol standing order does not apply to pregnancy-related hypertension. Obtain Medical Control for the obstetric indication, agent, dose, and blood-pressure endpoint.

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PROVIDER ACTIONS, SAFETY, AND RELEASE CONTROL

Provider-level actions

Level	Actions
EMT	Left lateral positioning when indicated; ABCs, glucose, hemorrhage assessment, seizure protection, oxygenation/ventilation support, and rapid transport.
AEMT	All EMT actions plus IV/IO access, AM-05 shock resuscitation, and authorized IM midazolam for an active seizure without delaying treatment for vascular access.
Paramedic	All preceding care plus cardiac monitoring, magnesium treatment, qualifying TXA, calcium-gluconate rescue for magnesium toxicity, and advanced resuscitation.

Safety checks

- Magnesium is the cause-directed eclampsia therapy. Use standardized midazolam only when immediate control of an ongoing convulsion is required.

- Vaginal bleeding may be absent with ectopic pregnancy or placental abruption. Shock disproportionate to visible bleeding requires urgent escalation.

- Do not rely on a normal ultrasound alone to exclude injury after maternal trauma; physician evaluation and appropriate fetal monitoring are required.

- Oxytocin is not carried on the Claiborne formulary and is not authorized under AO-03.

Linked pathways

Pathway	Use when
AO-01 Childbirth / Labor	Field delivery, postpartum care, and newborn coordination
AO-02 Newly Born	Immediate newborn resuscitation
AM-05 Hypotension / Shock	Hemorrhage, shock, or inadequate perfusion
UP-13 Seizure	Standardized active-seizure treatment

Source and release control

- American College of Obstetricians and Gynecologists. Gestational Hypertension and Preeclampsia, Practice Bulletin No. 222.

- Claiborne County EMS approved maternal-emergency, postpartum-hemorrhage, medication, and transport content.

- This is the controlled Claiborne source PDF and matches the approved Claiborne EMS app content.

- Document focused findings, interventions with dose/route/time/response, serial reassessments, destination decision, and notifications.